Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 0.1 | **Date:** 04-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A phase 2/3, multicenter, randomized, double-blind study to evaluate the efficacy, safety, pharmacokinetics and pharmacodynamics of oral ozanimod (RPC1063) in pediatric subjects with moderately to severely active ulcerative colitis with an inadequate response to conventional therapy **Short Title/ Acronym:** IM047-001 (Ozanimod in Pediatric UC) **Protocol Number:** IM047-001 / 2021-002308-11 **NCT Number:** NCT05076175 **Posting ID:** 903400739161514879 **Sponsor/Company Name:** Bristol-Myers Squibb **Investigational Product:** Ozanimod (RPC1063) **| Trade Name:** Zeposia **ATC Code:** L04AA38 **EMA Product Information:** Zeposia EPAR **Phase of Development:** Phase 2/Phase 3 **Trial Status:** RECRUITING (Currently recruiting participants) **Medical Monitor:** GSM-CT Representative (Bristol-Myers Squibb) / Principal Investigator (e.g., Basavaraj Kerur, MD)

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effectiveness, safety, pharmacokinetics, and pharmacodynamics of oral ozanimod (RPC1063) in achieving and maintaining clinical remission in pediatric participants with moderate to severe active ulcerative colitis (UC). **Secondary Objectives:** To evaluate long-term clinical response, mucosal healing, and safety over a 5-year open-label extension period.

2.2 Background & Rationale While ozanimod, a sphingosine-1-phosphate (S1P) receptor modulator, is approved for the treatment of moderate to severe UC in adults, there is a distinct clinical need to evaluate its appropriate dosing, safety, and efficacy in pediatric patients (ages 2-17). This study aims to provide targeted therapy for children who have had an inadequate response, loss of response, or intolerance to conventional treatments (such as aminosalicylates, corticosteroids, immunomodulators, or biologic therapy).

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Dose-comparison (e.g., High dose vs. Low dose) **Blinding:** Double-blind **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target N: 120 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 2021

Estimated Primary Completion: ~6 years from enrollment (1-year treatment + 5-year extension)

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Pediatric participants aged 2 to 17 years old. 2. Moderately to severely active Ulcerative Colitis (UC) diagnosed prior to the Screening Visit, with evidence of UC extending beyond the rectum, as determined by baseline endoscopy. 3. Has had an inadequate response, loss of response to, or is intolerant to at least 1 of the following treatments for UC: oral aminosalicylates, systemic corticosteroids, immunomodulators, or biologic therapy.

4.2 Exclusion Criteria 1. Diagnosis of Crohn's disease or indeterminate colitis. 2. Documentation of positive test for toxin-producing *Clostridium difficile*, or polymerase chain reaction examination of the stool. 3. Apheresis within 2 weeks of randomization. 4. History of or currently active primary or secondary immunodeficiency, or participants with known genetic disorders as a cause for colitis. 5. Other protocol-defined inclusion/exclusion criteria apply.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: For adolescents (aged 12-17): High dose (0.92 mg once a day) OR Low dose (0.46 mg once a day). Doses for younger children (aged 2-11) are scaled appropriately. **Route of Administration:** Oral. Administered via capsule, or a granule sprinkle formulation over food for younger participants unable to swallow capsules. **Duration of Treatment:** Up to 52 weeks (1 year) of double-blind treatment, followed by an optional long-term extension study lasting up to 5 years.

5.2 Concomitant & Prohibited Therapy Permitted: Stable background UC therapies (e.g., oral aminosalicylates) as defined by the protocol guidelines. **Prohibited:** Apheresis within 2 weeks of randomization; concurrent investigational biologics or immunomodulators not specified in the protocol.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Up to Day 0 | Informed Consent/ Assent, Medical History, Baseline Endoscopy, Stool Testing, Blood Draw, ECG, Eye Exam | | **Baseline** | Day 0 | Randomization, First Dose Administration, Diary Dispensation | | **Treatment** | Up to Week 52 | Efficacy Assessments, Safety Labs, PK/PD Sampling, Clinical Response Tracking | | **Extension** | Years 2-6 | Long-term Safety and Maintenance of Remission Monitoring | | **Follow-Up |**

Post-Treatment | 2 safety follow-up visits to ensure participant well-being and absence of ill-effects |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Proportion of participants achieving clinical remission at the end of the predefined induction/maintenance periods. **Measurement Method:** Clinical disease activity indices paired with central evaluation of endoscopy scores and stool frequencies.

7.2 Secondary & Exploratory Endpoints * Proportion of patients achieving and maintaining clinical response. * Pharmacokinetics (PK) and pharmacodynamics (PD) of oral ozanimod across pediatric age groups. * Long-term safety profile and tolerability of ozanimod over the 5-year extension period.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collected via patient/caregiver diaries, physical exams, routine blood/urine tests, ECGs, and eye exams to monitor S1P-specific safety signals. **Serious Adverse Events (SAEs):** Reported standardly within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** External, independent DMC tasked with reviewing unblinded safety and efficacy data, particularly for dose-finding in the younger (2-11 years) cohorts.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy endpoints; Safety Set for all participants receiving at least one dose of the investigational product. **Sample Size Justification:** Target $N=120$, powered to evaluate meaningful clinical remission and response differences between high/low dose cohorts and establish pediatric PK/PD profiles. **Handling of Missing Data:** Non-responder imputation (NRI) for categorical efficacy endpoints; mixed-model repeated measures (MMRM) for continuous variables.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine clinical site monitoring, centralized data review, and source document verification. **Audit Trail:** Standardized eCRF systems, centralized reading for endoscopic procedures to ensure unbiased assessment.

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11. Study Identifiers & Governance

Primary ID: {{IM047-001}} **Registry Number:** {{NCT05076175}} **Sponsor/Lead Organization:** {{Bristol-Myers Squibb}} **Study Title:** {{A Study Investigating Oral Ozanimod (RPC1063) in Pediatric Participants With Moderate to Severe Active Ulcerative Colitis}} **Official Regulatory Title:** {{A phase 2/3, multicenter, randomized, double-blind study to evaluate the efficacy, safety, pharmacokinetics and pharmacodynamics of oral ozanimod (RPC1063) in pediatric subjects with moderately to severely active ulcerative colitis with an inadequate response to conventional therapy}}

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12. Clinical Framework (Ulcerative Colitis Specific)

Primary Condition / Disease Areas: {{Moderately to severely active Ulcerative Colitis (UC)}} **Preferred Name:** {{Colitis}} **Preferred UMLS Name:** {{Pediatric Ulcerative Colitis}} **Semantic Type:** {{Disease or Syndrome}} **Definition:** {{Colitis refers to an inflammation of the colon and is often used to describe an inflammation of the large intestine (colon, cecum and rectum). Colitides may be acute and self-limited or chronic, and broadly fit into the category of digestive diseases.}} **Clinical Codes:** {{MeSH: D003092 | HPO: HP:0002583 | SNOMED: 64226004}} **Diagnosis Threshold:** {{UC extending beyond the rectum with inadequate response to conventional therapy}} **Medical Methodology:** {{Sphingosine-1-phosphate (S1P) receptor modulation}} **Optimization Target:** {{Clinical Remission and Mucosal Healing}}

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13. Study Procedures & Methodology

Management Pathway: {{Standard pediatric clinical pathway for treatment-refractory UC}} **Education Protocol (HCP):** {{Endoscopic scoring standardization and pediatric PK/PD assessment training}} **Education Protocol (Patient):** {{Caregiver/patient training for eDiary completion and granule sprinkle/capsule administration}} **Quality Audit Mechanism:** {{Centralized blinded review of endoscopies and electronic patient-reported outcomes (ePRO) tracking}}

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14. Observation & Follow-Up Schedule

Total Duration: {{Up to 6 years (1-year double-blind treatment + 5-year long-term extension)}} **Onsite Visit Cadence:** {{Protocol-defined intervals during 52-week double-blind phase, followed by extension visits}} **Remote Monitoring Frequency:** {{Interim phone contacts and diary compliance checks}} **Checklist Review Interval:** {{Daily symptom eDiary inputs with regular HCP reviews}}

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]
Lead Statistician	[Name]	____	[DD-MMM-YYYY]